

**Paying for Reliability: A Conditional Medicare Premium for Generic Cisplatin and  
Carboplatin**

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## **Paying for Reliability: A Conditional Medicare Premium for Generic Cisplatin and Carboplatin**

### **The Challenge**

In late 2022, FDA inspectors at an Intas Pharmaceuticals plant in Ahmedabad, India, found bags of torn and discarded manufacturing records, and an analyst destroying records by pouring acetic acid into a trash bin (U.S. Food and Drug Administration [FDA], 2023). That one plant supplied more than half of the U.S. market for generic cisplatin and carboplatin (Allen, 2023), two old, inexpensive chemotherapy drugs. No other supplier could make up the difference, and oncologists rationed. One Virginia clinic cut carboplatin to 60% of the optimal dose (Allen, 2023).

The shortage was the predictable result of a market that pays nothing for reliability. I defend an amended version of the Senate Finance Committee's May 2024 discussion draft, which offers a Medicare add-on payment for multi-year generic-injectable contracts. Wosińska and Frank (2024) criticized the draft because meeting contract standards does not guarantee a reliable supply. They proposed scoring contracts on delivery, adjusted for manufacturing-quality problems. I agree, and I add two conditions their proposal does not have: a verified alternate manufacturer that is actually shipping, and re-bidding when the contract ends.

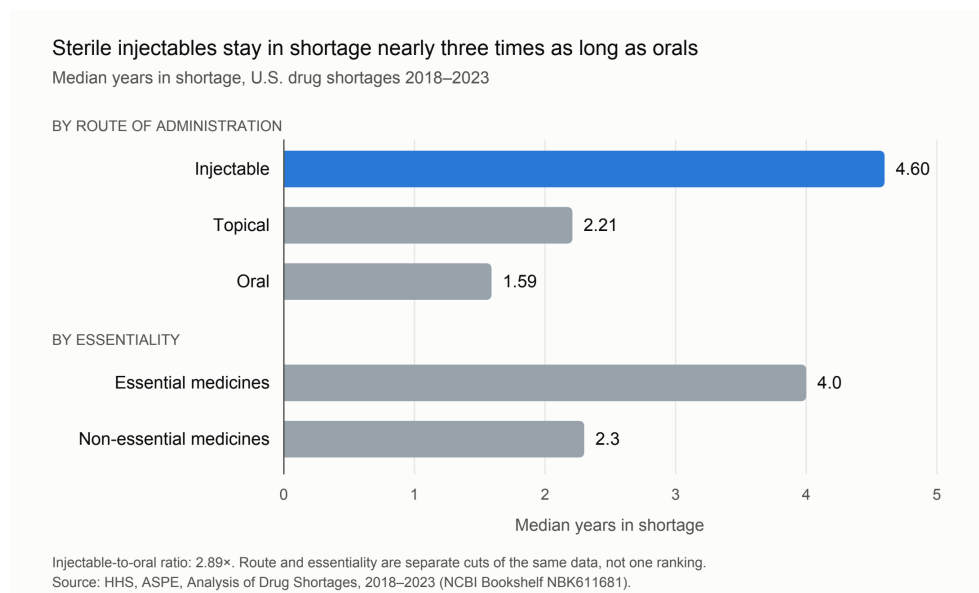
### **The Economics**

**Fixed costs and barriers to entry.** Bringing a generic injectable to market costs an estimated \$5.9 million to \$12.2 million and takes nearly twice as long as an oral drug to earn back (Holtkamp & Murphy, 2024, pp. 2, 4). So few firms hold spare capacity, and 40% of generic markets have a single manufacturer (Office of the Assistant Secretary for Planning and

Evaluation [ASPE], 2025). As Figure 1 shows, the median injectable shortage lasts 4.6 years, against 1.6 years for oral drugs.

**Figure 1**

*Median Years in Shortage, by Drug Type*



*Note.* Data from ASPE (2025).

**Price-inelastic demand.** A carboplatin dose is set by the treatment protocol regardless of price, so a supply cut leads to rationing and prices do not adjust to clear the market.

**Market structure.** Hospitals buy generic sterile injectables through group purchasing contracts that "commit manufacturers to a price but rarely carry a minimum quantity agreement," and they "primarily consider the price" because they cannot see drug quality directly (Wosińska & Frank, 2023). At 36 months, about 70% of generic injectable entries are underwater on net profit in the medium scenario, and 60–76% across scenarios (Holtkamp & Murphy, 2024, p. 7).

### **Analysis: What Would a Premium Have to Pay?**

I test whether a premium changes a manufacturer's decision to enter or stay in the market. Plant construction is outside the scope of this test. The cost of goods sold for generic injectables

runs 42% of revenue, leaving a 58% contribution margin (Holtkamp & Murphy, 2024, p. 2). A product covers its entry cost  $E$  when  $0.58 \times R \geq E$ , where  $R$  is revenue over 36 months. Treating a price uplift  $u$  as pure margin, the uplift needed to break even is:

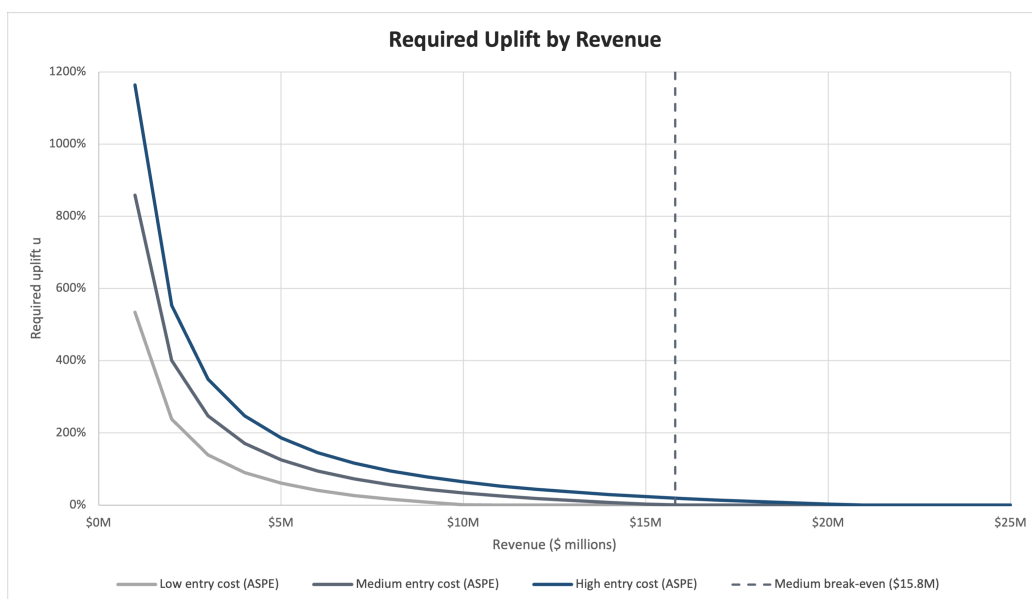
$$u = E / R - 0.58$$

As Figure 2 shows, in the medium scenario a product with \$15.8 million in 36-month revenue needs no uplift, one with \$10 million needs about 34%, and one with \$5 million needs about 125%. A flat premium sized for a mid-sized product falls far short for small ones, and small products are the old, cheap generics most likely to run short.

Cost versus benefit. Managing shortages cost U.S. hospitals an estimated \$894 million in labor in 2023 (Vizient, 2025), a floor estimate from a firm with a stake in the issue. Spread over about 323 active shortages (American Hospital Association [AHA], 2024), it is about \$2.8 million per shortage per year, or about \$13 million over a 4.6-year injectable shortage. The dollar premium a product needs is  $E - 0.58R$ , so it can never exceed the entry cost: \$9.2 million in the medium scenario and \$12.2 million in the high one. Per product, the premium comes in under the cost of a shortage, but not by much. Medicare would pay the premium on every enrolled product, but only some of those products would otherwise run short. The program pays off only if the premium per product, divided by the share that would otherwise run short, stays under about \$13 million. At the full \$9.2 million, that share has to be over 70%. For a product with \$10 million in revenue, the premium is about \$3.4 million and the share drops to about one in four.

**Figure 2**

*Required Price Uplift by Product Revenue, Three Entry-Cost Scenarios*



*Note.* The dashed marker shows the medium break-even revenue of \$15.8 million. Author's calculation from Holtkamp and Murphy (2024).

The estimate has limits. It ignores discounting, measures entry cost rather than plant construction, and uses generics launched from 2018 to 2021, not decades-old drugs like cisplatin, with only "suggestive evidence linking profitability to shortage risk" (Holtkamp & Murphy, 2024, p. 3). It also assumes the manufacturer receives the uplift, though the add-on is paid to providers.

### **Recommendation**

Medicare should pay hospitals and physicians a reliability add-on for contracts of three years or more, at meaningful volume, with quality-vetted sterile-injectable manufacturers, limited to drugs at high risk of shortage. It should pay only when a qualified alternate manufacturer is verified and actually shipping, the primary manufacturer has a clean FDA inspection record, and the contract is re-bid when it ends.

The Senate draft already requires contingency contracts with alternate manufacturers (U.S. Senate Committee on Finance, 2024), but nothing checks whether they work. I keep Wosińska and Frank's (2024) automatic delivery scoring and tighten their quality adjustment to a clean inspection record. The second supplier and re-bidding are my additions, so the payment rewards a backup source as well as on-time delivery. This also answers the pass-through limit. A provider is paid only when its manufacturers ship, and they will not keep shipping unless enough of the payment reaches them, which Wosińska and Frank call "self-regulating" (2024, p. 5). CMS would score from existing FDA inspection and delivery data, so hospitals file nothing new.

The Hamilton Project alternative. Its \$2 billion in partly forgivable loans lowers the cost of building a plant (Wosińska & Frank, 2023), but the breakeven shows that the problem is small products not selling enough to repay their entry cost. A multi-year volume commitment addresses that directly.

**Main objection.** A premium could entrench incumbents and be run by the same purchasing structures that pushed generic prices below the cost of reliable supply, protecting margins without adding capacity. But a premium that pays only when a second source is shipping rewards redundancy, so an incumbent with no working backup earns nothing. Re-bidding also limits how long any incumbent keeps the payment.

### **Conclusion**

The cisplatin shortage came from high entry costs, inelastic demand, and purchasing that places no value on a second supplier. Supporting entry is affordable for mid-sized products and expensive for small ones. Set against the labor cost of a shortage, the premium pays for itself only narrowly, and only if enough of the money goes to products that would otherwise run short. That is why the add-on has to be targeted and conditional.

## References

- Allen, A. (2023, June 21). *Drugmakers are abandoning cheap generics, and now US cancer patients can't get meds*. KFF Health News. <https://kffhealthnews.org/health-industry/drugmakers-are-abandoning-cheap-generics-and-now-us-cancer-patients-cant-get-meds/>
- American Hospital Association. (2024, April 12). *ASHP reports record high number of drug shortages*. AHA News. <https://www.aha.org/news/headline/2024-04-12-ashp-reports-record-high-number-drug-shortages>
- Holtkamp, N., & Murphy, S. (2024, December). *An examination of the return on investment of generic injectable prescription drugs* [Issue brief]. Office of the Assistant Secretary for Planning and Evaluation, U.S. Department of Health and Human Services. <https://www.ncbi.nlm.nih.gov/books/NBK611650/>
- Office of the Assistant Secretary for Planning and Evaluation. (2025, January 10). *Analysis of drug shortages, 2018–2023* [Data brief]. U.S. Department of Health and Human Services. <https://www.ncbi.nlm.nih.gov/books/NBK611681/>
- U.S. Food and Drug Administration. (2023, July 28). *Warning letter: Intas Pharmaceuticals Limited (MARCS-CMS 652067)*. <https://www.fda.gov/inspections-compliance-enforcement-and-criminal-investigations/warning-letters/intas-pharmaceuticals-limited-652067-07282023>
- U.S. Senate Committee on Finance. (2024, May 1). *Preventing & mitigating generic drug shortages* [Discussion draft summary]. [https://www.finance.senate.gov/imo/media/doc/050124\\_sfc\\_drug\\_shortages\\_discussion\\_draft\\_one\\_pager.pdf](https://www.finance.senate.gov/imo/media/doc/050124_sfc_drug_shortages_discussion_draft_one_pager.pdf)

Vizient. (2025, June). *The hidden cost of drug supply chain disruptions*. <https://vizientinc-delivery.sitecorecontenthub.cloud/api/public/content/0fed86e17e654732ba642464400e713f>

Wosińska, M. E., & Frank, R. G. (2023, June). *Federal policies to address persistent generic drug shortages* [Policy proposal]. The Hamilton Project, Brookings Institution.  
<https://www.hamiltonproject.org/publication/policy-proposal/federal-policies-to-address-persistent-generic-drug-shortages/>

Wosińska, M. E., & Frank, R. G. (2024, July 5). *Comments on Senate Finance Committee draft legislation to combat prescription drug shortages*. Brookings Institution.  
<https://www.brookings.edu/articles/comments-on-senate-finance-committee-draft-legislation-to-combat-prescription-drug-shortages>